جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه صادق محمد عويس
المحافظة : الجيزه **الحي / المدينة :** الصف **العنوان :** العطيات الصف الجيزة
رقم البطاقة : 27802192100721 **رقم التليفون :** 01113807237 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 19-02-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه صادق محمد عويس
National ID: 27802192100721

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف روسي

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

الهيئة المصرية لصحة مصر

03/26/2026

MDT FORM

Name:	Fatma Sadek Mohamed Ewis	**MRN:**	2170
Age:	48 Y	**ID:**	27802192100721
Menopausal Sta	premenop - cth induced ammenorrhea	**Phone:**	01153674672
NRS SCORE:		**Marital Status:**	MARRIED WITH 5 OFFSPRING
BMI:		**Family History:**	-VE (unconfirmed breast lump on mother's side)
CO Morbidity:	medically free	**Reason Of MDT:**	POST NACT

Clinical Data:

BX: RT BR IDC GII

ER: 0/8 **PR:** 0/8 **HER2:** 3+ **KI67:** 40%

Sonomammography:
ACR B
RIGHT BREAST UOQ LESION WITH MICROCALCIFICATION
NON-SPECIFIC LN
T2 N0 M0

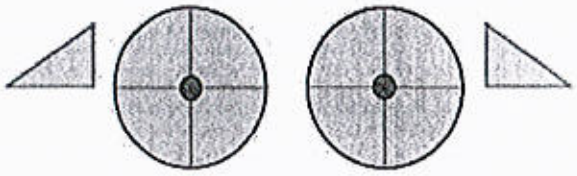
Mets Workup:
CT CAP FREE
multiple rt renal stones

History:
clips inserted
for TCHP x6 followed by BCS + SLNB

Recommendations:
pt received 18 weeks TC and NO ANTIHER2 TTT, refused by MOH multiple times
last chemotherapy on 12/2
CLINICALLY NO PALPABLE TUMOUR

MDT Outcome & Plan
For Rt BCS+ SLN then adj ttt according to operative pathology

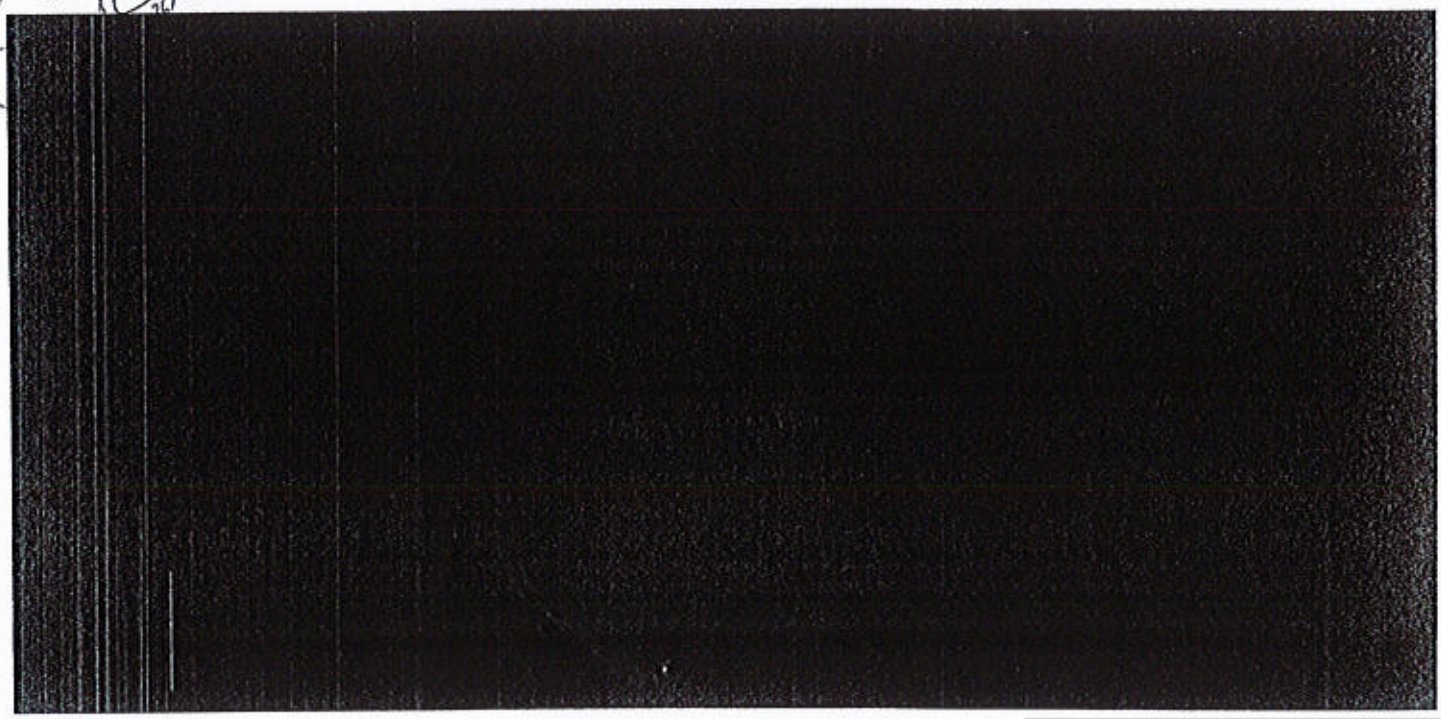
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لاشين د/ ندي حسن	د/ خالد السعدي	د/ هاني مختار			







National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name: فاطمه صادق محمد عويس **MR. :** 2170	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with premenopausal right-sided breast cancer, clinical stage T2, node-negative, estrogen-receptor positive, progesterone-receptor positive, HER2 2+ with SISH amplification. Surgical clips were placed. Baseline tumor measurements were weight 102 kg, height 150 cm, and a performance status of 2. The patient was started on a 15-week adjuvant regimen of weekly Taxol 150 mg and Carboplatin AUC 2. During week 16 the patient developed grade 2 peripheral neuropathy. Anti-HER2 therapy had not yet been administered. The patient has completed 16 weeks of the Taxol-Carboplatin regimen to date.

The same clinical details were reiterated, and it was noted that the patient had not yet received anti-HER2 therapy. A request for adjuvant PHESGO for week 18 was denied by the Ministry of Health for the surgical multidisciplinary team. The patient therefore remains without anti-HER2 treatment. The patient has completed 16 weeks of the Taxol-Carboplatin regimen to date.

Adjunctive PHESGO was not approved by the Ministry of Health; consequently, the plan is to initiate Herceptin subcutaneous administration after completing the required paperwork and obtaining the necessary approvals, including echocardiography, bone scan, and CT imaging.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

جوبستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السيدات المصرية تحت مظلة

MDT FORM 15/8/2025

Name		MRN	
Age		ID	
Menopausal status		Phone	
IRS SCORE		Marital status	
MI		Family history	
Comorbidity		REASON OF MDT	

Clinical Data

BI:	PR:	HER2:	Ki67:

Mammography:

Lets Workup:

Recommendations:

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	[illegible]	Request No.	: 13009222
Patient No.	27802192100721	Request Date	: 23/04/2025
Path No.	1465/25	Verification Date	: 10/05/2025
Sex	Female	Financial Category	: [illegible]
Age	47 Y	Department	: Out Patient
Referring Doc		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 23/04/2025.

Gross Examination:
- Multiple greyish white tissue cores up to 1.2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (485), Ki67 clone (30-9), Calponin clone (EP798Y) & SISH.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
- Ductal carcinoma in situ: Not detected.
- Perineural invasion: Not detected.
- Angioinvasion: Not detected.
- Microcalcifications: Yes.
- Additional pathologic findings: Necrosis.
- IPT (Positive control showed appropriate reactivity):
- Calponin: Negative.
- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: Moderate incomplete membrane reaction in >10% of tumor cells (score 2+).
- Ki67: 45% nuclear reaction of tumor cells.
- SISH: Amplified.

Average Her2 copy No: 6.1
Her2/CEP17 ratio: 3.2

Diagnosis:

Verified by:
Dr.Menan Bossam

CAP
Road No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt
عمارة رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر - الدور الأول - طريق رقم (4)

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient ID: 56664	Patient Name Eng'sh: Fatmah Sadek Mohamed
Age: 46 Y 7 M 0 D	2025/04/13 01:25:17 PM Report Date

Women's Health Outreach Program

Name: fatema sadek Mohamed

Date: 13-04-2025

Bilateral Full field Digital Mammography

Clinical summary: 46yrs /o female with right lump sensation from 3 weeks

Technique

Mediolateral oblique and CC views

Detailed Findings:

Right breast:

- Fairly dense mammary parenchyma (ACR b)
- UOQ ill defined dene lesion with asymmetry
- U/S shows at 11 o'clock zone a irregular heterogenous hypoechoic solid lesion seen measure(26x18mm away from skin by 5mm and from nipple by 15 cm it contain macrocalcifications with penetrating blood vessels by color doppler
- Normal skin/nipple/areolar complex.
- Non specific axillary lymph nodes detected.

Left breast:

- Fairly dense mammary parenchyma (ACR b)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Normal skin/nipple/areolar complex.
- Non specific axillary lymph nodes detected.

Impression:

Left breast: BIRADS 1

Right breast: BIRADS 4 UOQ palpable malignant looking lesion for biopsy

Best regards,

Dr . Marwa M.Hassan

Mammogram
[[DOCTYPE:MAMMOGRAM]]

tient Name	[illegible]	Request No.	: 13009222
tient No.	: 27802192100721	Request Date	: 23/04/2025
n No.	: 1465/25	Verification Date	: 10/05/2025
x	: Female	Financial Category	: [illegible]
e	: 67 Y	Department	: Out Patient
ferring Dep	:	Registered by	: [illegible]

Pathology Report

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37, 45% nuclear reaction of tumor cells,
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rage Her2 copy No: 6.1
2:CEP17 ratio: 3.2

nosis

Verified by :
Dr.Mosaad Hossam

ing Date : 21/05/2025 16:13:05 Page 1 of 1

Printing By : [illegible]

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient ID: 86664
XR 3 Views Age:
Patient Name: Fatimah Sadek Mohamed
2025/04/13 01:25:12 PM Report Date

Women's Health Outreach Program

Name: fatema sadek Mohamed
Date: 13/04/2025

Bilateral Full field Digital Mammography

Clinical summary: 46yrs /o female with right lump sensation from 3 weeks

Technique

Mediolateral oblique and CC views

Detailed Findings

Right breast

- Fairly dense mammary parenchyma (ACR b)
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Impression:

Left breast: BIRADS 1

Right breast: BIRADS 4 UOQ palpable malignant looking lesion for biopsy

Best regards,
Dr. Marwa M.Hassan

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name: Fatma Sadek Mohamed Owais	Date : 21/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 47 Year(s)
Medical ID: 1838/2025	Pat. ID: 110610
690	

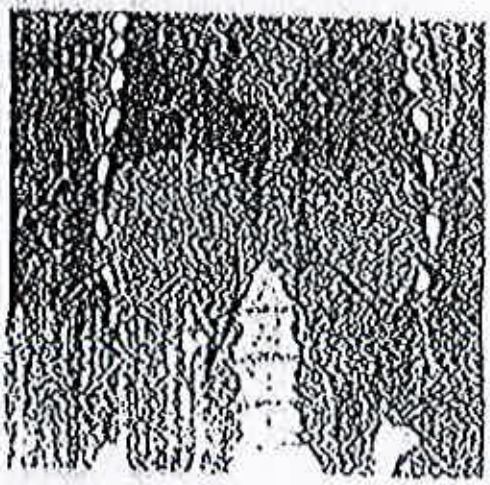
MULTISLICE NON CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multiplanar axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction.

FINDINGS:

(I) Chest:

- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.



(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density with no focal lesions on non contrast enhanced basis.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, left kidneys.
- Two right renal pelvis radiodense stones are seen measuring 12x7.5 mm and 10x6.2 mm in diameter and 12 HU attenuation with mild pelvicalyceal changes.
- Right renal lower calyceal stone is seen measuring 8.6x6.5 mm in diameter and 970 HU in attenuation.

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستافروسي

- Mild perinephric fat stranding is noted.
- Pelvic phleboliths are noted.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retrocrural, para-aortic or pelvic lymphadenopathy on non contrast-enhanced basis.
- No ascites.

OPINION:

Normal CT study of the chest with no pulmonary metastatic nodules.

Multiple right renal stones with mild hydronephrosis.

Dr. Sara Mahmoud, MD